



1. General information

1.a Applicants

In case an applicant has more than one affiliation, please mention this in the table.

Name*	Institution	Department	Email	Tenured position / Tenure-track/ Other	EMC clinician / researcher	Role in Flagship
Jason Pridmore	EUR	ESHCC	pridmore@eshcc.eur.nl	Tenured Associate Professor		Main Lead
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Hein Raat	EMC	Public Health	h.raat@erasmusmc.nl	MD, PhD, MBA Full Professor	Researcher	Lead
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Corresponding Lead: *Jason Pridmore*

Planned start date of seed funding: *01/08/2022*

Primary Convergence theme: *Technology Supported Transitions in Health(care)*

Secondary Convergence theme (if applicable): *Human Centered Technology and AI for Health*

Tertiary Convergence theme (if applicable): *Improving Health Journeys*

Keywords: *contextual trust, technology, promotion, prevention, collaboration, communication*

1.b Summary (max 300 words)

Trust.

This Flagship proposal is about a focused consideration of trust in relation to health and technology that builds on a contextual approach. Drawing on the work of Nissenbaum (2004) and others that discuss 'contextual integrity', we re-conceptualise and make tangible a 'contextual trust' approach as a Convergence Flagship.

Norms and conceptions of trust vary across cultures, time, and locations (Hansen, 2019; Mouslim et al., 2020; Shahbari et al., 2020; Thorne & Robinson, 1988; Zhao et al., 2019). The maintenance or violation of trust is dependent upon these contextual normative accounts. This involves power dynamics, information disparities, mistrust, concern and hope, all while human lives are at stake (Jaiswal & Hilkitis, 2019). The complex dynamic between the actors involved calls for an interdisciplinary and collaborative effort that incorporates technical, social, and organisational perspectives (Kramer & Tyler, 1995; LoCurto & Berg, 2016; Xu et al., 2014).

Our 'contextual trust' approach operationalizes the different trust contexts within intersections of health, technology and healthcare. Different but overlapping trust factors are embedded in these intersections across critical issues and moments. The SWITTCCH Flagship looks at these trust factors in conjunction with critical processes within critical moments of care. These critical processes include an understanding of Trustworthy Communication Practices (Chaudri et al., 2020), Digital Technology Development (Ding et al., 2021), and Digital Tools within Interpersonal and Organisational Interactions (Doekhie et al., 2019).

We examine these processes in light of critical care moments, following the IOM framework of healthcare (O'Connell et al., 2009). Specifically, we focus on Promotion and Prevention (e.g. municipality communication platforms), Treatment - (e.g. within hospital, rehabilitation, care facilities), and Maintenance - (e.g. home-based, self-monitored, after care, telecare). Understanding the intersections of trust factors, critical processes, and critical care moments provide a blueprint for ways to create effective and trustworthy technologies within health and social care.

1.c Public Summary (max 300 words)

The development and successful integration of new mediated technologies into health and social care provisions is predicated on trust. It is the fabric that makes the potential for new innovations possible and it is the lack of trust which dooms well intended and seriously innovative processes to failure. The SWITTCCH Flagship is based on the assumption that trust is essential as a premise to the development of new technologies rather than an afterthought. Instead of beginning with a clever solution and ending with trying to build trust into newly developed technologies and systems that solve critical issues, the SWITTCCH Flagship will start from the basis of establishing and forming trust and embeds this into our collaborative developments.

The SWITTCCH Flagship conceptualises trust as contextual. This contextual approach is borrowed from Nissenbaum (2004) who rejects a one-size-fits-all conceptualization of data privacy. Rather, all elements and aspects of human life occur through and are governed by context-specific norms. Protection for privacy is then tied to a context-specific approach to information gathering and sharing. In this Flagship, this understanding of contextuality is connected to the concept of trust. Drawing from the IOM framework of healthcare (O'connell et al., 2009), this project sets out to analyse contextual trust in light of critical moments of care: Promotion and Prevention, Treatment, and Maintenance. For each, the Flagship indicates different foci:

- Communication - broadscale engagements increasing capacities for information dissemination and public awareness;
- Digital Technology Development - design and solution-based engineering for health concerns and issues;
- Interpersonal and Organisational Interactions - face-to-face and team-based situations that involve health and social care decisions.

The result of the SWITTCCH Flagship is a sustainable, interdisciplinary research delivering a coherent research framework built around different critical moments, critical processes, and trust factors that, put together, form a conceptualization of contextual trust.

2. Relevance to Convergence Health & Technology mission

(50%) — max. 2500 words for 2.a to 2.e

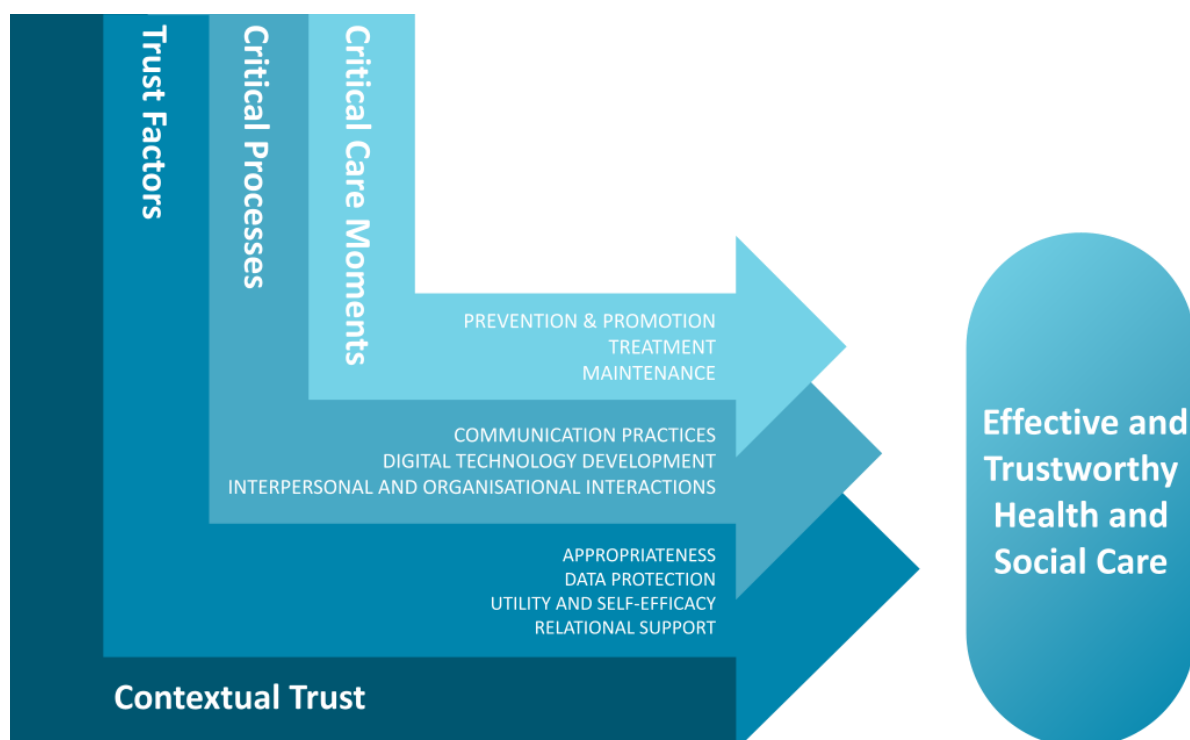
- *The Flagship program contributes to the Convergence Health & Technology mission (1)*
- *The Flagship program objectives fit with the guidance per theme provided by the theme leads, and/or fit with more than one theme of Convergence Health & Technology (2)*
- *The Flagship program merges approaches and insights from distinct disciplines to reach its goals (3)*
- *The staff granted by this call actively brings together different disciplines within the program, and creates visibility and attracts talents, partners, and funding for the program (4)*
- *The Flagship program has a clear view on long term sustainability after the 5-year central funding period (5)*
- *The envisioned results of the Flagship program have the potential for a significant contribution to healthcare, society and/or have scientific impact which has the potential for long-term societal impact (6)*
- *The Flagship program provides opportunities for BSc/BA and/or MSc/MA students of the participating institutes to participate in (interdisciplinary) projects (7)*

- The proposal defines the pathway of how this contribution can be realized. The pathway includes concrete research and/or impact pathway (societal and/or economical) valorisation strategies (8)

2.a Contribution to H&T mission and fit with H&T theme(s)

The SWITTCH Flagship is based on the assumption that trust is essential to the development of new technologies, departing from the assumption that the implementation of new technological developments is primarily hindered by new designs or improved techniques. Rather, trust is at its basis relational. In the context of technology and health, trust must be woven through the interactions of people with the development and implementation of new technological solutions. Success and effectiveness is based on an understanding of and assessments within real-world contexts that bridge theory and practice. This approach connects directly with the mission of the Health and Technology Convergence. To make use of technology to achieve lifelong health and well-being, to ensure a more equitable distribution of healthcare, to increase preventative practices and to tailor treatment all requires a significant degree of trust that is the focus of this Flagship proposal.

Often trust is approached through quantitative models that focus solely on user-acceptance. We know that norms and conceptions of trust depend on contextual normative accounts. The complex dynamic between the actors involved calls for an interdisciplinary and collaborative effort that incorporates technical, social, and organisational perspectives. This builds on and leads to a more focused understanding of 'contextual trust'.



The SWITTCH flagship focuses on contextual trust factors which are addressed and operationalised across **critical processes** within **critical care moments**. These trust factors include:

- *Appropriateness*: Is the technology suitable and applicable within that context?

- *Data Protection*: Does the technology ensure secure and private data handling within that context?
- *Relational Support*: Does the technology facilitate positive relational engagement between actors within that context?
- *Utility and Self-efficacy*: Will the technology being used be effective and feasible for use in that context? Is the technology understandable and viable for users?

The first critical process *communication practices* addresses broadscale engagements which increase capacities for information dissemination and public awareness for a public audience. *Digital technology development* engages in design and solution-based engineering for health concerns and issues. *Interpersonal and organisational interactions* focuses on face-to-face (in)formal care, social support decisions, and team-based situations. We specifically address how trust factors intersect with the critical care moments *promotion and prevention* (e.g. municipal communication platforms), *treatment* (e.g. within hospital or rehabilitation and care facilities), and *maintenance* (e.g. home-based, self-monitored, after care, or telecare). Together, an understanding of the intersections of trust factors, critical processes, and critical care moments provide a blueprint for ways to create effective and trustworthy technologies within health and social care. Using a multidisciplinary perspective, the SWITTCHE flagship operationalizes this contextual trust framework within six interconnected work packages elaborated in section 3.b.

Within the Health & Technology Convergence call, the SWITTCHE Flagship addresses three core themes. *First*, this Flagship contributes to improving health journeys by resituating trust as the essential starting point. By breaking down health care journeys into critical care moments, the SWITTCHE Flagship ensures that all components are involved within the larger framework of contextual trust. *Second*, the Flagship is committed to the exploration of technology-based transition within healthcare. A significant increase in digital technologies within and in relation to the healthcare domain has changed perceptions and experiences of healthcare. The SWITTCHE Flagship critically evaluates how health and social care are being shaped by technological interventions, ranging from digital communication, to monitoring devices, to team based applications. These technology-based transitions need to embed an understanding of contextual trust to create effective and trustworthy health and social care. *Finally*, the Flagship is devoted to the development of human-centred technology and AI for health. Technological innovations like AI bring along opportunities and challenges. These technologies have a need for human-centred and user-led design to embed privacy and security. It is pivotal to evaluate and execute trust in the design process of digital health technologies. This design process is central within SWITTCHE, including the use of decentralised EDGE AI to support and improve health and social care for vulnerable individuals.

2.b Synergy between disciplines

The SWITTCHE Flagship is at its core interdisciplinary. It draws on disciplines embedded in the Erasmus School of History, Culture and Communication (ESHCC), Erasmus School of Health Policy and Management (ESHPM), and Erasmus School of Philosophy (ESPhil), the TU Delft departments of Industrial Design and Engineering (IDE) and Technology, Policy and Management (TPM) and

Public Health from Erasmus MC. These applicants have the ambition to form a truly sustainable, cross-disciplinary scientific programme and research group combining distinct frameworks with varying insights around the core-issue of contextual trust to improve health and well-being for all.

Our synergies are clearest in relation to our Critical Processes. Within *communication practices*, ESHCC members focus on the role that effective and engaging communication has in creating perceptions and understandings of technological trustworthiness in health care processes and social care issues. This connects with a focus on the social determinants of health, a focus of our colleagues from ESPhil. Effective communication is important in the transmission of information from health care providers to health care users and essential to successful interpersonal interactions within the formal health care providers teams, institutions, and organisations. This connects with the expertise of the researchers of ESHPM on teams and (in)formal care. Our colleagues at EMC and the Gemeente Rotterdam provide the context and users who engage with communicative practices of health care, simultaneously producing and representing recipients of communicative practices.

In relation to *digital technology development*, the Flagship focuses on an understanding of how to embed trustworthiness into technological design as critical. Led by our partners at TU Delft, this work focuses on effective design development (IDE) and innovative edge computing techniques (TPM). This fits within the broader understanding of ESHCC and ESPhil that technological advancements should be user-led and human-centred. Our practitioners within Public Health at EMC and the Gemeente Rotterdam are key here as well. This work strives to develop technologies that embed an appropriate and secure handling of data within user-led designs in ways that will not lose effectiveness in solving critical health related issues. The adaptation of digital tools within organisational structures provides a rich research context for our ESHPM partners.

In the last critical process, *interpersonal and organisational interactions*, our ESHPM partners examine the implementation of new digital technologies within health care processes. An additional focus is on issues of healthcare transfer from formal to informal caregivers with the implementation of new digital support tools as part of a user centric design focus of ESHCC and IDE. This focus will be practically realised within the health care teams and projects of our EMC partners as well as the social support projects carried out by Gemeente Rotterdam.

2.c Long term sustainability plan

The SWITTCH Flagship is situated within broader pan-European research developments that contribute to solving critical health issues and promotes innovative new technological advancements. Specifically, Work Package 6 focuses on practical, specific, and tangible outputs that will contribute to further long-term research and projects to build towards sustainable convergence focused on contextual trust within health and social care.

We take for granted several factors. *First*, concerns about trust and technology will continue past the timeline for this Flagship. Our team situates itself as a crucial node in these discussions for research in the forthcoming decades, both in terms of practical solutions and academic literature.

We have begun this by successfully executing the TRESKA, Valuecare, and SPATIAL projects, as well as the forthcoming IANUS project (amongst several other related projects), making central different aspects of trust across multiple disciplines. *Second*, digital technologies and new forms of data gathering, analysis, and use will continue long beyond our Flagship funding timeline. Our research and work will set precedents for and define appropriate trustworthy approaches for how to work with such technological developments. *Third*, this Flagship will set the stage for increasing cross institutional cooperation in the realm of education. We have identified opportunities for partnerships within programmes, offering courses that expand the breadth of knowledge within different disciplines. For instance, this Flagship sees the potential for a successor to the already successful SecureHospitals MOOC that engages healthcare workers into an awareness of cybersecurity issues in their workplace. We identify potentials for educational related activities using our relationships and resources to make cross institutional cooperation increasingly possible and embedded in our institutions. We build this upon our experience and knowledge within our current projects, further elaborated in section 3.d.

Risks and Contingency Plan

As with any Flagship proposal, there are several identified risks particularly in relation to its long-term sustainability. A first risk is the possibility of reduced sources of funding in the forthcoming years, as our partners primarily focus on government rather than corporate funding. However, each of our current projects make use of industrial partners that can be leveraged for future development work. Additionally, this risk is limited given the increased attention and importance placed on health and the focus on ensuring appropriate digital tool developments for health.

Another risk is our focus on the concept of contextual trust itself. Some may not understand the importance of this concept and focus on more concrete digital tools. Since the SWITTCHE Flagship differs from the other Convergence Flagships in this, there may be a risk that these complexities are not easily marketable in concrete terms. However, the SWITTCHE Flagship is focused on demonstrating acceptability within our Postdoc and PhD based case studies. We are confident that this will show how moving trust to the centre rather than the periphery of digital health tool implementation is critical. For instance, tangible digital tools such as smart devices that incorporate user-centric EDGE AI designs can be more easily commercialised because of their trust-based starting point. Additionally, the complexity of contextual trust also leads to opportunities for our Flagship to act in a consultant capacity in a range of settings.

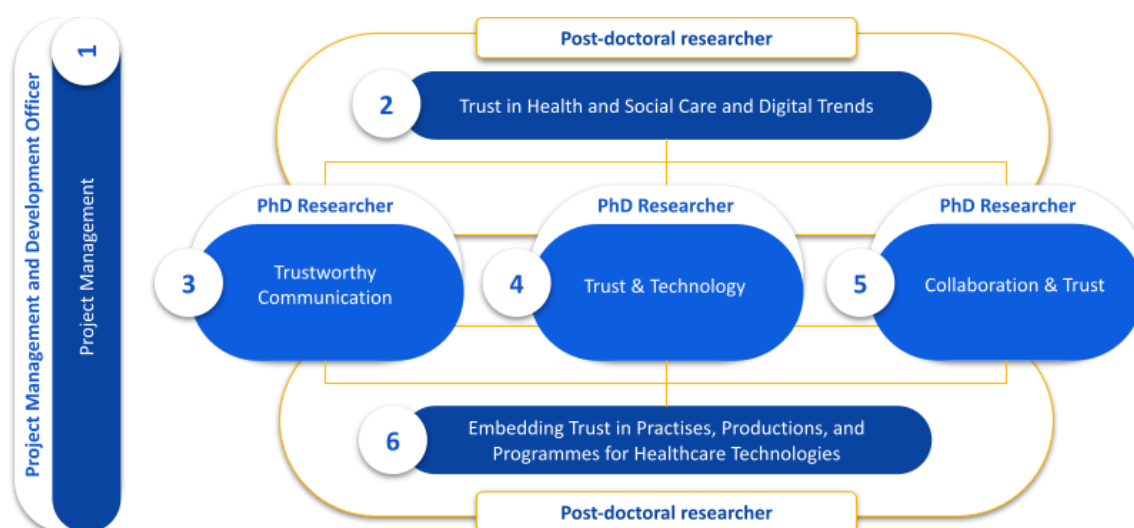
The SWITTCHE Flagship sees bureaucratic hurdles as a significant risk towards cross-institutional cooperation. It will need cooperation across the convergence process to function best. While we are experienced with this within EU and NL projects, the Convergence initiative differs from this, for example even determining the budgetary processes for this proposal. However, this risk will not be faced alone, and cooperation particularly in relation to research and education development will be explored in detail. Additional risks and contingencies are further identified and addressed in section 3b.

2.d Visibility, attracting talents and new partners

The SWITTCH flagship is situated at the intersection of healthcare, communication sciences, technology design, organisation management, and policy research. Students, staff, and stakeholders from these disciplines are brought together and are enabled to forge collaborative ties within interdisciplinary research environments. The broad range of collaborative initiatives across the Netherlands and Europe are evident of the interest that trust in healthcare, technology, and wellbeing generates.

The consortium members already possess a high degree of visibility, given their previous work within previous and existing projects such as TRESKA, SPATIAL, ValueCare, and SecureHospitals. The demonstrated capacity for Consortium members to attract and participate in different forms of funding is showcased by their track record (see section 4.a). By incorporating Edge-AI, SWITTCH is situated at the cutting-edge of new technology developments and AI, attractive for new health and technology developments. This is further substantiated by the strong publication record by Flagship members and their established contacts within the domain of health and social care, health policy and communication professionals. Strong ties with professionals in engineering, design, and technological innovation further facilitate the consortium's capacity to attract funding. Current collaborations include large scale industrial partners and SMEs.

Our interdisciplinary approach is vital to attract people who want to address societal issues in a coordinated manner, in conjunction with the knowledge and skills of others. As such, we define clear potentials for early career researchers to participate in relation to the Flagship's work packages. An overview of the positions and projects that SWITTCH will generate is provided in the diagram below.



The SWITTCH Flagship will be active in attracting young and ambitious talent and aims to involve students and early career researchers during its trajectory. The following positions are envisioned:

1. Project Management and Development Officer
 - Central node amongst research staff, leads, applicants, and departments

- Identify with consortium leads promising calls and potential partnerships
- 2. Post-Doctoral Researcher I
 - Identifies multi-disciplinary approaches to trust within health and technology
- 3. PhD Researcher: Trustworthy Communication
 - Communication and public engagement among healthcare professionals, audiences, and technologies
- 4. PhD Researcher: Trust and Technology
 - Human-centric design and digital infrastructures for privacy and security, embedding trust into the development of ethical technologies
 - Focused on dynamics of human interaction, digital tools, and trust.
- 5. PhD Researcher: Collaborations and Trust
 - Interactions within institutions, organisations, and teams
 - Formal and informal care connections
- 6. Post-doctoral researcher II
 - Embedding trust in practices, productions, and programmes for healthcare and wellbeing technologies
 -

2.e Long term societal impact

The SWITTCH Flagship is structured around six work packages (see 3.c for our work plan). In specific, Work Package 6 provides four concrete outputs. These have been developed with a focus on cross-disciplinary partnership. The SWITTCH flagship develops trust based principles and concepts empirically determined and tested in practice. The summation of this Flagship is its wider integration into developing projects and critical resources. These include the following outputs:

Output 1: Tailored Trustworthy Guidelines Recommendation Platform

This creation of a tailored and automated recommendation platform can be used in the design and development of new technologies within health and social care. This platform will consist of more general guidelines and tailored definitions for ensuring trustworthy and effective digital tools and techniques. Medical institutions and care facilities, patients, and medical practitioners can refer to this platform and utilise the theory-to-practice translations as an important means for more effective, tailored, and reliable healthcare for all.

Output 2: Privacy Supporting EDGE-AI Tools

Drawing on the tools developed within work package 4, this collects together key processes that need to be a part of trustworthy healthcare tool development such as using an EDGE-AI approach. These tools will start from the assumption that a trustworthy approach is essential to the development of technology especially with privacy and security issues for data collection, storage, and analysis.

Output 3: Trust Support for other Convergence Flagships

Considering trust as essential to successfully integrate technological innovations into the health domain, with other Flagships in this Convergence initiative and beyond, SWITTCH makes use of its

expertise to support the broader initiative. We will critically engage with others to increase user-acceptance and trusted data use.

Output 4: Educational offerings

Given our multidisciplinary perspectives, we will establish key courses that connect across faculties and institutions. To begin, the development and integration of relevant courses within various represented programmes on issues of trust within healthcare practices will be developed. Here we first focus on our differing master programmes. Key will be our ESHPM partners, but other master degrees such as Digitalisation, Surveillance and Societies (ESHCC), Medicine (Erasmus MC), or Management of Technology (TPM) will be able to take SWITTCH based elective courses. Expansion of this offering into a bachelor minor (short term) or the development of an independent master programme (long term) are also ambitions.

3. Flagship proposal (25%) max 2000 words for 3.a to 3.d

- The Flagship program contributes to excellent Convergence Health & Technology research, at the forefront of international research (1)
- The Flagship program is both innovative as well as challenging in its research and approach (2)
- The Flagship program has a clear strategy to engage students in its research plan (3)
- The Flagship program has a clear aim and long-term work plan, including, if applicable, a specification of required facilities and a data-and infrastructure plan. Coherence within the program is clearly demonstrated (4)
- The Flagship program aligns with or builds on (inter)national research agendas in this field, fit with the (inter)national research agendas but has a “high risk, high gain” main objective that is thoroughly explained (5)
- The Flagship program is feasible in terms of research performed, staff and budget needed (6)
- The Flagship program has a concrete plan for attracting external funding, which should result in at least the same amount as the central funding over the 5-year period (7)

3.a Ambitions

The SWITTCCH Flagship reiterates that human behaviour plays a crucial role in successfully adopting new technologies. This Flagship is built on the premise that developing technologies for healthcare and social well-being needs to understand contextual trust as a key, central element; that digital tools must be rooted in a contextual trust approach for development, implementation, and interpretation. This process brings with it new potentials for better healthcare but also brings significant challenges to address. There has been a positive impact on and great potential for digital technologies to create more equitable health and social care, but there is a crucial need to evaluate how trust is integrated in digital health technologies.

The overarching concept of contextual trust for this Flagship creates significant complexity. In comparison to other flagships, this work may be seen as less concrete. As such, we set particular ambitions for this project connecting contextual trust in practice.

Ambition 1: Change the way health and social care is communicated

This Flagship sees the need to move towards engaging communication practices in the domain of health and social care. In order for institutional communication methods to be effective, trust needs to be embedded throughout the communication process. Broad societal issues can be addressed including the spread of medical misinformation and mistrust in traditional healthcare, something tremendously problematic during the pandemic (Sauer et al., 2021).

Ambition 2: Change the way technology design and development is done

Our capacity to develop highly complex digital devices drawing on machine learning is growing incrementally. This is driven by access to increasing amounts and a diversity of data. However, concerns about privacy, data security, and data protection have become increasingly critical and need to be addressed appropriately. The SWITTCCH Flagship will improve the way in which technology design and development is conducted by adopting an iterative design approach sensitive to these issues and incorporating a human-centric and user-led focus. We will build towards an innovative framework that embeds, evaluates, and executes trust in the design process of digital health technologies.

Ambition 3: Change the way in which digital tools interface with health and social care teams and (in)formal support

This Flagship engages with the challenges brought along by the adoption of digital tools into (in)formal care teams. In these contexts, team members devise and determine workarounds for technical issues, share critical information within and beyond formal digital tools, and shape the organisational contexts in which digital devices both reinforce and disrupt healthcare practices. Based on the notion of contextual trust, we overcome (some of) these limitations by adopting a user-centric approach to account for the radically changing ways in which digital tools interface with health and social care teams and (in)formal support.

3.b Aims and feasibility

Following our ambitions, the aims of SWITTCH are threefold. First, the project will *generate an understanding of the factors that together constitute contextual trust*. Second, the project will *study key moments in healthcare and social wellbeing through the lens of contextual trust*. These key moments are: communication practices, digital technology development, and interpersonal and organisational interactions. Third, *these factors will be studied in the light of critical care moments* that emerge from existing literature: prevention & promotion, treatment, and maintenance (O'Connell et al., 2009). Following these aims, the SWITTCH flagship will create an ecosystem for collaborative efforts to target and tackle societal issues through trust based technology design and development for health and social care. This approach, given both its research trajectories and incorporation into education, is suitable for students and academic staff and researchers alike.

The feasibility of the Flagship is dependent on a number of factors. Cross institutional developments without already pre-existing determinants such as found in most grant organisations (NWO, ZonneMW, European Commission), may lead to differentiated approaches that limit collaboration. Overcoming these even within our institutions has proven to be a problem. Several additional feasibility factors have already been identified in addition to those listed in section 2c.

First, the Flagship is dependent upon key team members: as SWITTCH is a collaborative effort, team members may shift positions during the course of the project. However, any risk this poses is mitigated by the structure of this Flagship which ensures progress is not reliant on individual team members.

Second, the financial capacity of the partners raises other concerns related to the complexity of an interdisciplinary, interfaculty project. However, all partners involved are experienced with the dynamics of collaborative research ecosystems. The proposed Flagship is not reliant on financial support of any single institutional entity.

Third, the technological tools might fail to deliver the anticipated results, due to technicalities or insufficient connections to relevant stakeholders. Yet the team members already have

consolidated effective relationships with a number of companies in this area that provide more direct access to potential clients.

Finally, the inability of the different departments involved to work together may hinder developing sustainable education offerings. This is a particularly significant concern in the development of convergence educational programmes currently directed at EUR by Prof. Dr. Evert Stamhuis and the lead applicant, a Vice Dean of Education at EUR. Some of these concerns are being addressed but limitations in this area - alignment of academic calendars, financing, opening up courses - create particular feasibility challenges for this Flagship.

3.c Work plan

The SWITTCCH Flagship has a coherently developed plan for implementation represented in our work package structure. This is illustrated and described as follows:



Work Package 1: Project Management

The first work package focuses on effective cross institutional and faculty collaboration. Effective project management is needed given the different procedures and cultures that differ institutionally and within faculties. The primary work here is to set out cooperative process development for the case studies and research work, managing the planning for the different work packages.

Work Package 2: Trust in Health and Social Care and Digital Trends

The increase in digital technologies within and in relation to healthcare has changed perceptions and experiences. Most prominently, the COVID-19 crisis showed the need to more effectively communicate in a more trustworthy manner and ensure the minimization of misinformation. Likewise, critical issues in data security and concerns about privacy and confidentiality have made for care recipients to be reticent to allow their data to be made more broadly accessible. Building on our research projects on trust (TRESKA, IANUS), values (VALUCARE), and data security (SPATIAL, SecureHospitals), the SWITTCCH Flagship evaluates current trends in how health and social care are shaped by technological interventions, setting out conceptual and practical approaches for a multidisciplinary understandings and processes.

Deliverable 2.1: Overview how digital trends affect trust in technology in health and social care (report).

Deliverable 2.2: Design conceptual model and multidisciplinary approach for WP 3, 4, and 5.

Work Package 3: Trustworthy and Engaging Communication Practices

New digital technologies have changed how the public understands and responds to health and social care issues. The SWITTCHE Flagship sees trustworthiness as a dynamic process and a function of effective and engaging mediated communication processes. Understanding the communication processes and perceptions of differing interfaces of health related practices is vital for how trust is built. This work package focuses first on communication practices as critically important for the broadscale engagement among healthcare professionals, their audiences and relied upon technologies. By weaving trust throughout these practices, this work package proposes targeted and more effective communication strategies. With the municipality of Rotterdam, we will use its platforms to communicate inhabitants (“Wijk aan Zet” and “Zorgzaam010”) and the public health program of the municipality “Gezond010” to focus on preventive interventions through eHealth applications. Trust in these applications is essential for local stakeholders, like general practitioners, who need to promote and support their use.

Deliverable 3.1: Scientific paper on social media as source of healthcare information, focusing on particular case studies, eg. cosmetic surgery and disability.

Deliverable 3.2 Literature review on how communication tools can be implemented to enhance engagement in technologies in health and social care.

Deliverable 3.3 Co-design communication prototype for case study “Wijk aan Zet” (Neighbourhoods in Charge), evaluation examining trust and end user engagement, guidelines for communication strategy and redesign.

Work Package 4: Digital Technology Development and Ethical Design

This work package designs and assesses digital technologies supporting trust through privacy and security within user-led designs. Personal behavioural data plays a critical role in developing and executing ethical digital healthcare solutions. Behavioural data reveals critical insights that drive human-centred design processes by encouraging designers to connect with users and unravel hidden patterns and outliers. These interactions are vital for addressing the relevant challenges with the appropriate solution. However, this data also drives products’ intelligence, which should not require a trade-off of privacy or security. We offer healthcare solutions improving lives without data compromises by engaging users in the iterative design and development process. This intertwines two novel approaches: Edge AI and Data-Centric Design.

EDGE AI puts recipients of care in control of how they are monitored and what data is released by preserving data sharing controls in the locations in which they are produced. Data-Centric Design brings personal data in a participatory design process to generate rich, contextual insights that inform the design of appropriate design solutions. This combined approach demonstrates a human-centred, data-centric, iterative design process that fosters trust in the context of health

and social care for independent living of vulnerable persons. It provides reliable, trusted and informed monitoring tools that clients/patients and their networks can control themselves in a secure, ethical and privacy-preserving manner.

Deliverable 4.1: Development of trustworthy health tools using the technology of EDGE-AI.

Deliverable 4.2: Development of Data-Centric Design tools to support responsible, ethical and data-led participatory design in the healthcare context

Deliverable 4.3: Evaluation of trustworthy EdgeAI and Data-Centric Design tools, usability, trust and engagement in case studies.

Deliverable 4.4: Guideline for developing trustworthy health tools.

Work Package 5: Trustworthy Digital Tools for Interpersonal and Organisational Interactions

Technological interventions have the capacity to shape, facilitate, or impede interpersonal interactions between health care professionals, support staff, patients, and informal caregivers. This work package examines the dynamics of human interaction and digital tools and the role of trust in this relationship. Trust among professionals in health & social care and trust in the technology will be linked to the usability and effectiveness of the technology and the role behaviour of professionals working in teams. Further we study trust relationships between formal care, informal care and patients in telemonitoring projects and their impact on safety.

Deliverable 5.1: Scoping literature on how digital tools affect interaction between patients and (in)formal caregivers

Deliverable 5.2: Scientific paper on trust between professionals within a digital platform (case study 1) and its consequences.

Deliverable 5.3: Scientific paper on the enhanced role of patients and informal caregivers in providing information through digital tools (case study 2).

Deliverable 5.4: Guideline for developing interaction-friendly health tools.

Work Package 6: Embedding Trust in Practises, Productions, and Programmes for Healthcare Technologies

This work package focuses on four final outputs previously discussed in section 2.e: Long Term Societal Impact.

3.d Alignment of the program with relevant national and international research agendas

The SWITTC Flagship aligns with and builds on both the national Dutch research agenda and broader European projects focused on continued collaboration to better healthcare for all. The Dutch Research Agenda (NWA) focuses on the structural strengthening of connections between science and society (projects IANUS following TRESKA). It fits within varying themes of the NWA including AI, racism and discrimination, ground-breaking medical research, and healthy ageing. The unique contribution of the SWITTC Flagship is its starting point of contextual trust. Healthcare is taken as a broad framework and includes municipal prevention and promotion programmes, organisational practices, patient-doctor interactions, and potentials of maintenance care at home. The project is part of AI theme due to its development of EDGE AI digital tools for the creation of secure and private data handling of patients. The Flagship is uniquely situated in

relation to international Horizon projects, with members demonstrated a capacity not only to win these grants but lead them. These projects have always been based on creating significant collaborative ambitions that result in positive societal developments in a 'high risk, high reward' approach.

4. Consortium (25%) max 2000 words for 4.a to 4.d

- The Flagship program is driven and executed by a well-organized team of scientists from all three institutions and at least 4 departments or faculties in total. In exceptional cases, it is possible to form a Flagship program with only two leads from two institutions, but this should be clearly motivated (1)
- Members of the consortium have a track record of excellence in academic research, translation of scientific results into clinical application or societal impact (2)
- Members of the consortium have a significant track record in engaging and supervising BSc and MSc students in research projects (3)
- Members of the consortium have demonstrated to be able to attract external funding, thereby enhancing the programs sustainability after the 5 years of central funding (4)
- There is a demonstrable fit between the aims and objectives of the program and the expertise of the consortium members (5)
- End users (private and/or public sectors) are involved from an early start and team members are well-connected to external partners in the ecosystem (6)
- Governance should be clearly described and is organized in such a way that it facilitates collaboration and optimally makes use of the expertise of all participants (7)

4.a Track record of the consortium members

dr. Jason Pridmore, PhD is an Associate Professor in the department of Media and Communication at EUR and the Vice Dean of Education at ESHCC. He is the coordinator of the Horizon 2020 funded TRESKA project focusing on trustworthy communication practices. He is the principal investigator of the NWO funded Mapping Privacy and Surveillance Dynamics in Emerging Mobile Ecosystems project. He is the Data Security and Exploitation manager for the H2020 funded BIM-SPEED project and Ashvin project and contributor to the Horizon 2020 funded SecureHospitals project and SPATIAL project. Jason Pridmore and his research team brings to this Flagship *a focus on user centric design, user acceptance, privacy and a practice centred approach.*

dr. Aaron Ding, PhD, is a tenured Assistant Professor at the Dept. Engineering Systems and Services at TUD. He is the coordinator of the Horizon 2020 funded SPATIAL project (5M) and PI for Horizon Marie Curie ITN APROPOS project (4M) to develop approximate edge AI solutions. He has received Nokia Foundation Scholarships, Best Paper Awards from ACM EdgeSys 2019 and ACM SenSys CCIoT 2020, Best-in-Session Presentation Award at IEEE INFOCOM 2018, and Best CCR of ACM SIGCOMM 2014. Aaron Ding brings to this Flagship *development capacities and world renowned expertised in the areas of Edge AI and privacy preserving data practices.*

Prof. dr. Hein Raat is a full professor at the Department of Public Health at EMC. He is the coordinator of the Horizon 2020 funded ValueCare project and the European Union Health Programme SEFACproject. He previously led the European Union Health Programme UHC2.0 (Urban Health Centres Europe), and the ZonMw consortium CIKEO. Prior to joining Erasmus Medical Center, he worked as a medical specialist and manager in public health. He is an active member of the 'academic working places' CEPHIR and ST-RAW and chair of the quality committee at the department of Public Health. He chairs a research line of circa 20 members based on

competitive funding. Hein Raat brings to this Flagship *collaborations with multiple public stakeholders and a focus on potentials for technology integration in public health.*

dr. Martina Buljac, PhD, is a tenured Assistant professor of Organisational Behaviour at ESHPM. Her work focuses on human resource management within health care teams, with a focus on measuring and improving team effectiveness. She has published in a broad range of healthcare related journals and has developed an instrument to measure team effectiveness in order to improve effectiveness by deploying interventions. She leads EUR's efforts on the ZonMW funded project Safe Transfer, and is involved in several developing healthcare project proposals focusing on effective teamwork and management. Martina Buljac brings to this Flagship *a focus on effective teamwork within healthcare contexts and the integration of informal care with formal care practices.*

dr. Marleen de Mul, PhD, is an assistant professor (EUR) has extensive experience in researching the development, testing and implementation of technologies for ageing (AAL, AHA, social robotics) and Health Information Technology. Research has been published in the Journal of Medical Internet Research, International Journal of Social Robotics, and European Geriatric Medicine. Methodological expertise in co-design, qualitative research, and action research, which has resulted in a methodology for agile co-creation of robotics for ageing (LIFE). She has supervised over 80 thesis projects at Bachelor and Master level and three PhD students who work on co-design and information infrastructures. Marleen brings to this flagship a focus on the normalisation of technology in care practices.

dr. Yogi Hendlin, is an environmental philosopher and public health scientist. Hendlin is assistant professor at the Erasmus School of Philosophy, and core faculty of the Dynamics of Inclusive Prosperity Initiative at Erasmus University Rotterdam, as well as research associate in the Environmental Health Initiative at the University of California, San Francisco. Hendlin's interdisciplinary and transdisciplinary research projects tackle major questions in philosophy of biology, environmental philosophy, and political philosophy. Hendlin's epistemological inquiries into public health follow the via negativa of agnotology, comprehending the systemic transmission of ignorance.

Prof. dr. Hub Zwart, has been the dean of Erasmus School of Philosophy since 2018. In 2004 he became director of the Centre for Society and Genomics (CSG) funded by the Netherlands Genomics Initiative (NGI) and established at his department. He is editor-in-chief of the Library for Ethics and Applied Philosophy (SPRINGR, LOET) and of the journal Life Sciences, Society and Policy (Springer). The focus of his research is on philosophical and ethical issues in the emerging life sciences with a focus on genomics and post-genomics fields such synthetic biology, nanomedicine and brain research, but he is also interested in research integrity and academic authorship. Special attention is given to the use of genres of the imagination (novels, plays, poetry) in research and education.

dr. ir. Jacky Bourgeois, is assistant professor of Data-Centric Design at the Faculty of Industrial Design Engineering. He develops tools and methods for Responsible Data-Centric Design (RDCD),

that is accounting for data subject privacy, data collection and processing transparency, involvement of stakeholders, data confidence and design process traceability.

dr. Amy van Grieken, is an assistant professor at Erasmus Medical Centre. Her main research interest is in the development and evaluation of (preventive) interventions that promote public health with regard to health and well-being. The focus here is on integrated social and health care for children, parents, and older people, supported by E-health and value-based health care. In all projects, practice-based research collaborating with cities and (health care) professionals is key.

4.b Expertise of the consortium members in relation to the aims and objectives of the program

As mentioned in 3.b, three main aims of the programme can be identified that together have the underlying goal of a better understanding and more effective technology within health care through the notion of contextual trust. First, the aim of the SWITTCHE flagship is to change the way health and social care is communicated in order to ensure better engagement between receivers and providers of health care as well as within healthcare teams. The expertise brought forward from the applicants of ESHCC and ESHPM will be essential to improving trustworthy communication on health care and social support. Second, there is a need for a more data-sensitive and human-centred development of digital tools. This development will be expanded due to the expertise of our colleagues at TU Delft. Finally, the SWITTCHE project aims to change the way in which digital tools interface with health and social care teams and (in)formal support through a focus on user-engagement. The extensive knowledge from the consortium members of ESHPM will enrich this aim. Conclusively, the EMC applicants as well as our partners at Gemeente Rotterdam provide the case studies and users that ensure practical contexts for the development of our theoretical knowledge into sustainable, functional practical outputs.

All consortium members have a broad understanding of trust from different areas of expertise. Bringing together these perspectives provides a joint, sustainable breeding ground for further academic research and practical outputs. The colleagues from ESHCC and ESHPM have broad knowledge on communication, organisational structures, management, interpersonal interactions within health care processes. Furthermore, the applicants from ESPhil understand the social determinants of health and further highlight the need for a social science approach to digital healthcare technologies. Finally, the colleagues from EMC and ESHPM bring the medical perspective. This complementarity of members' expertise ensures that together, the SWITTCHE flagship realises the importance of trust within the intersections of critical processes and critical care moments.

4.c Involvement of partners

The main partner involved in the SWITTCHE project is the Gemeente Rotterdam. The municipality plays a vital role in the larger framework of public health and social support, making use of online platforms and digital tools. The municipality has set up "Gezond010" a project prioritising a healthy and active lifestyle for all inhabitants of the city of Rotterdam. This project will also focus on eHealth applications where the focus is twofold. On the one hand, the eHealth applications provide the possibility of improved support, information provision, and health care service. Thus,

the eHealth applications support the municipality in its efforts to engage citizens and provide effective health care and social support. On the other hand, eHealth applications provide the receiving users and citizens with more agency over their health journeys and lifestyles. Furthermore, the municipality uses several communication platforms to communicate directly with its citizens. These include, but are not limited to, “Wijk aan Zet” and “Zorgzaam010”. Throughout the entire Flagship the Gemeente Rotterdam will be involved as a full partner, providing expertise, case studies, concrete projects, and users to translate the theoretical research into practical contexts. Concretely, the two main partners at the Gemeente Rotterdam are Wilma Jansen and Denis Wiering. The former is connected to EMC directly as a colleague of prof. Hein Raat in her role on effective youth policy, focused on public health, in the city of Rotterdam. The latter is a programmanager on Impact and Innovation for the Gemeente Rotterdam and is connected to Erasmus University, Rotterdam School of Management. The municipality will provide the context and users for different case studies within the different work packages. Having Gemeente Rotterdam as a partner makes it possible for the SWITTCH project and its applicants to situate itself as the crucial node for research in the forthcoming decades and provides the possibility to contribute to academic literature as well as practical, engaging solutions. Additionally, the SWITTCH project will build on successful existing research projects that engage in the issue of trust (TRESKA, IANUS), values (VALUCAARE), and data security (SPATIAL, SecureHospitals).

4.d Governance, organization and collaboration strategy of the consortium

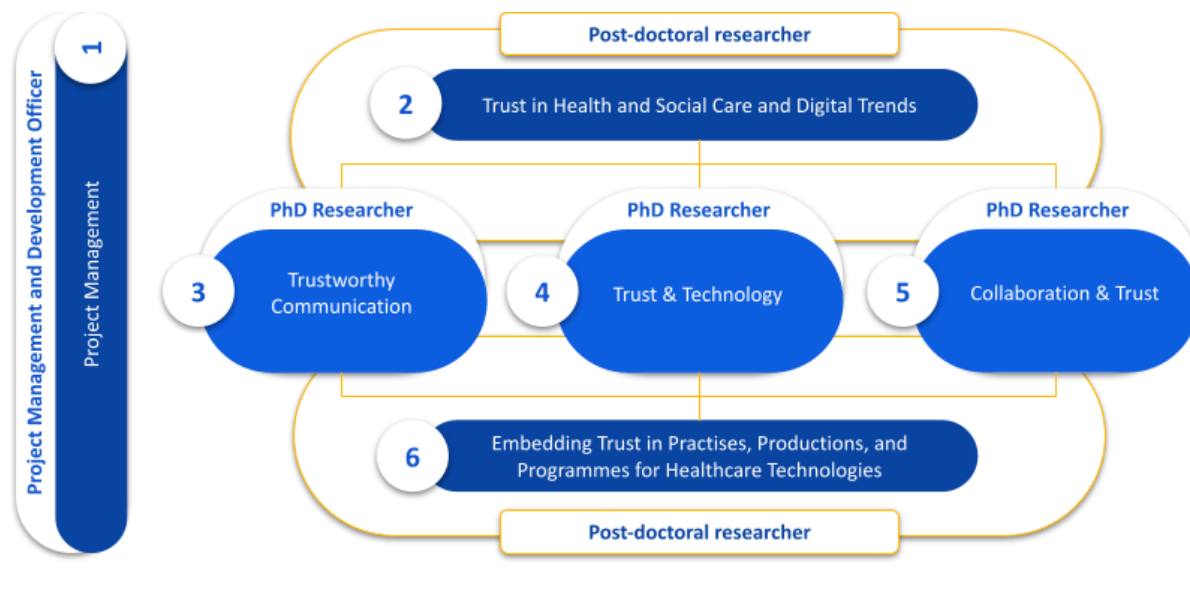
The SWITTCH Flagship will be organised around a structure and design of independent but collaborative work packages. EHSCC will be the main lead of this consortium alongside the project developer and manager (WP1) who will coordinate the postdoctoral researcher and key leaders from EMC. All the other key-applicants from EHSCC, ESHPM, and TU Delft respectively will be the coordinator of a PhD candidate where postdocs from EMC will interface.

EMC will be the lead of work package 2, with co-partners from ESHPM and ESPhil, who focus on the fundamentals of health care within organisational structures and the focus on the societal influences of public health. This work package is focused on the construction of a general framework which will provide the fundamentals of the other work packages. Special attention is paid to innovations that are not reductive to single disease pathways, or short-term relief or treatment alone, but would be seen ‘ecologically’ as part of a larger suite of public health interventions that intentionally design society according to a biopsychosocial model of health. Here, technological interventions are ancillary to the more social-material interventions and remediations, which do not require private investment, specialised access, nor additional education.

Work package 3 will be directed by ESHCC in a close collaboration with EMC due to its focus on public health and social support communication, operating in conjunction with EMC and Gemeente Rotterdam. TU Delft will oversee work package 4, in close conjunction with the user group of EMC. The expertise of the faculties Technology, Policy, and Management as well as Industrial Design Engineering will be made use of. ESHPM will take the lead in work package 5. EHSCC will provide expertise for engaging communication within interpersonal and organisational

interactions Furthermore, again, EMC will provide the practical contexts for case studies to be explored within WP5.

Finally, work package 6 is the second EMC-based work package and will be led by a postdoctoral researcher overseeing the three work packages, working towards outputs. As noted in section 2.e, this work package aims to generate four concrete outputs which are produced through collaborative efforts by EMC and TU Delft. The outputs comprise a tailored and automated recommendation platform, privacy supporting Edge-AI tools, the integration with other Convergence Flagships, and educational offerings. The research expertise of key personnel from TPM and EMC serves as a guideline for creating the platform and trustworthy Edge-AI implementations. Educational modules and courses are then implemented to create a sustainable ecosystem in which the mission of Convergence is embedded.



Flagship:	Supporting Well-being through Innovation and Trustworthy Technology for Care and Health		
Flagship acronym:	SWITCH		
Naam Penvoerder:	Jason Pridmore		
Startdatum en looptijd van het project:	1-sep-22	60 months	
Totaal project budget:	€		3.675.970.34

Loonkosten:								2022		2023		2024		2025		2026		2027		Totaal		Totaal per partner
Partner die de kosten maakt	Functie	Aantal fte	Startdatum	Einddatum	Inschaling	Maandbedrag	Kosten	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	
Onderzoekorganisaties																						
EUR-ESHCC / Convergentie	PhD	0,80	1-1-2023	31-12-2026	UNI - p	€ 3.793	€ 218.490			€ 54.622,51		€ 54.622,51		€ 54.622,51		€ 54.622,51		€ 54.622,51		€ 218.490,05	€ 0	
EUR-ESHCC / Matching	PhD	0,80	1-1-2023	31-12-2026	UNI - p	€ 3.793	€ 218.490			€ 54.622,51		€ 54.622,51		€ 54.622,51		€ 54.622,51		€ 54.622,51		€ 218.490,05	€ 0	
EUR-ESHCC / Convergentie	Project manager	0,80	1-1-2023	31-12-2027	UNI - 10	€ 4.238	€ 305.108			€ 61.021,60		€ 61.021,60		€ 61.021,60		€ 61.021,60		€ 61.021,60	€ 61.021,60	€ 305.108,00	€ 0	
EUR-ESHCC / Matching	Project manager	0,80	1-1-2023	31-12-2027	UNI - 10	€ 3.817	€ 274.824			€ 61.021,60		€ 61.021,60		€ 61.021,60		€ 61.021,60		€ 61.021,60	€ 61.021,60	€ 305.108,00	€ 0	
EUR-ESHPM / Convergentie	PhD	1,00	1-1-2023	31-12-2026	UNI - p	€ 3.793	€ 273.113			€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 273.112,56	€ 0	
EUR-ESHPM / Matching	PhD	1,00	1-1-2023	31-12-2026	UNI - p	€ 3.793	€ 273.113			€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 273.112,56	€ 0	
TU Delft / Convergentie	PhD	1,00	1-1-2023	31-12-2026	UNI - p	€ 3.793	€ 273.113			€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 273.112,56	€ 0	
TU Delft / Matching	PhD	1,00	1-1-2023	31-12-2026	UNI - p	€ 3.793	€ 273.113			€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 68.278,14		€ 273.112,56	€ 0	
Erasmus MC / Convergentie	Postdoc	1,00	1-1-2023	31-12-2027	UMC - 11	€ 4.868	€ 438.162			€ 78.792,00		€ 83.420,00		€ 87.512,00		€ 91.922,00		€ 96.516,00		€ 438.162,00	€ 0	
Erasmus MC / Matching	Postdoc	1,00	1-1-2023	31-12-2027	UMC - 11	€ 4.868	€ 438.162			€ 78.792,00		€ 83.420,00		€ 87.512,00		€ 91.922,00		€ 96.516,00		€ 438.162,00	€ 0	
							€ 0													€ 0	€ 0	
							€ 0													€ 0	€ 0	
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							€ 0													€ 0	€ 0	
							€ 0													€ 0	€ 0	
							€ 0													€ 0	€ 0	
Ondernemingen																						
Onderneming 1						€ -	€ 0													€ 0	€ 0	
Onderneming 2						€ -	€ 0													€ 0	€ 0	
Onderneming 3						€ -	€ 0													€ 0	€ 0	
Onderneming 4						€ -	€ 0															

[illegible]

3.	Kosten van gebruik van machines en apparatuur						2022		2023		2024		2025		2026		2027		Totaal		Totaal per partner	
	Partner die de kosten maakt	Machine / apparatuur			Prijs per gebruikseenheid	Gebruiks eenheden	Kosten	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind			
	Onderzoeksorganisaties																		€ 0	€ 0	€ 0,00	
	Organisatie 1																		€ 0	€ 0	€ 0,00	
	Organisatie 2																		€ 0	€ 0	€ 0,00	
	Organisatie 3																		€ 0	€ 0	€ 0,00	
	Organisatie 4																		€ 0	€ 0	€ 0,00	
	Ondernemingen																					
	Onderneming 1																		€ 0	€ 0	€ 0,00	
	Onderneming 2																		€ 0	€ 0	€ 0,00	
	Onderneming 3																		€ 0	€ 0	€ 0,00	
	Onderneming 4																		€ 0	€ 0	€ 0,00	
	Overige (private) partners																					
	Naam 1																		€ 0	€ 0	€ 0,00	
	Naam 2																		€ 0	€ 0	€ 0,00	
	Naam 3																		€ 0	€ 0	€ 0,00	
	Naam 4																		€ 0	€ 0	€ 0,00	
	Totaal						€ 0												Totaal kosten machines en apparatuur		€ 0	
	4.	Aan derden verschuldigde kosten						2022		2023		2024		2025		2026		2027		Totaal		Totaal per partner
		Partner die de kosten maakt	Naam derde, omschrijving kosten				Kosten	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind			
Onderzoeksorganisaties																		€ 0	€ 0	€ 0,00		
Organisatie 1																			€ 0	€ 0	€ 0,00	
Organisatie 2																			€ 0	€ 0	€ 0,00	
Organisatie 3																			€ 0	€ 0	€ 0,00	
Organisatie 4																			€ 0	€ 0	€ 0,00	
Ondernemingen																						
Onderneming 1																			€ 0	€ 0	€ 0,00	
Onderneming 2																			€ 0	€ 0	€ 0,00	
Onderneming 3																			€ 0	€ 0	€ 0,00	
Onderneming 4																			€ 0	€ 0	€ 0,00	
Overige (private) partners																						
Naam 1																			€ 0	€ 0	€ 0,00	
Naam 2																			€ 0	€ 0	€ 0,00	
Naam 3																			€ 0	€ 0	€ 0,00	
Naam 4																			€ 0	€ 0	€ 0,00	
Totaal						€ 0												Totaal kosten derden		€ 0		
5.		Publicatie, reis- en verblijfkosten						2022		2023		2024		2025		2026		2027		Totaal		Totaal per partner
		Partner die de kosten maakt	Naam derde, omschrijving kosten				Kosten	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind			
	Onderzoeksorganisaties																		€ 0	€ 0	€ 0,00	
	Organisatie 1																		€ 0	€ 0	€ 0,00	
	Organisatie 2																		€ 0	€ 0	€ 0,00	
	Organisatie 3																		€ 0	€ 0	€ 0,00	
	Organisatie 4																		€ 0	€ 0	€ 0,00	

6.

	in Euro		2022		2023		2024		2025		2026		2027		Totaal		Total per partner
			in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	
Onderzoeksorganisaties																	
1 Erasmus MC / Convergentie					€ 85.292		€ 152.420		€ 156.512		€ 98.422		€ 103.016		€ 595.662	€ 0	€ 595.662
2 Erasmus MC / Matching					€ 85.292		€ 152.420		€ 156.512		€ 98.422		€ 103.016		€ 595.662	€ 0	€ 595.662
1 EUR-ESHCC / Convergentie					€ 123.144		€ 123.144		€ 123.144		€ 123.144		€ 63.522		€ 556.098	€ 0	€ 556.098
2 EUR-ESHCC / Matching					€ 123.144		€ 123.144		€ 123.144		€ 123.144		€ 63.522		€ 556.098	€ 0	€ 556.098
6 EUR-ESHPM / Convergentie					€ 75.778		€ 75.778		€ 75.778		€ 75.778		€ 2.500		€ 305.613	€ 0	€ 305.613
3 EUR-ESHPM / Matching					€ 75.778		€ 75.778		€ 75.778		€ 75.778		€ 2.500		€ 305.613	€ 0	€ 305.613
4 TU Delft / Convergentie					€ 75.778		€ 113.278		€ 113.278		€ 75.778		€ 2.500		€ 380.613	€ 0	€ 380.613
4 TU Delft / Matching					€ 75.778		€ 113.278		€ 113.278		€ 75.778		€ 2.500		€ 380.613	€ 0	€ 380.613
Totale bijdrage onderzoeksorganisaties	€ 0	€ 0	€ 719.985	€ 0	€ 929.241	€ 0	€ 937.425	€ 0	€ 746.245	€ 0	€ 343.075	€ 0	€ 343.075	€ 0	€ 3.675.970	€ 0	€ 3.675.970
Ondernemingen																	
4 Onderneming 1															€ 0	€ 0	€ 0
5 Onderneming 2															€ 0	€ 0	€ 0
6 Onderneming 3															€ 0	€ 0	€ 0
... Onderneming 4															€ 0	€ 0	€ 0
Totale bijdrage ondernemingen	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0
Overige (private) partners																	
7 Naam 1															€ 0	€ 0	€ 0
8 Naam 2															€ 0	€ 0	€ 0
9 Naam 3															€ 0	€ 0	€ 0
... Naam 4															€ 0	€ 0	€ 0
Totale overige bijdrage	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0	€ 0
			in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind	in cash	in kind			Totaal
Totaal budget per jaar			€ 0	€ 0	€ 719.985	€ 0	€ 929.241	€ 0	€ 937.425	€ 0	€ 746.245	€ 0	€ 343.075	€ 0			€ 3.675.970
																	Funding Flagship
Totaal budget			€ 0		€ 719.985		€ 929.241		€ 937.425		€ 746.245		€ 343.075		€ 3.675.970		

Erasmus MC / Convergentie	€ 595.662
EUR-ESHCC / Convergentie	€ 556.098
EUR-ESHPM / Convergentie	€ 305.613
TU Delft / Convergentie	€ 380.613
Totaal Convergentie	€ 1.837.985
Erasmus MC / Matching	€ 595.662
EUR-ESHCC / Matching	€ 556.098
EUR-ESHPM / Matching	€ 305.613
TU Delft / Matching	€ 380.613
Totaal Matching	€ 1.837.985



**Gemeente
Rotterdam**

Maatschappelijke Ontwikkeling
Beleidsadvies WMO & Jeugdhulp

Bezoekadres: Het Timmerhuis
Halvemaanpassage 90, Rotterdam
Internet: www.rotterdam.nl

Van: W. Jansen
Telefoon: 14 010
E-mail: w.jansen@Rotterdam.nl

Onderwerp: Letter of intent

Datum: 14 maart 2022

Retouradres: Postbus 70012, 3000 KP ROTTERDAM

Per email verzonden

JASON Pridmore
Erasmus University
Department of Media and
Communication

Dear dr Jason Pridmore ,

Trust is a recurring theme in the interaction of the municipality and its inhabitants.
The SWITTCH consortium will provide us with insights to support well-being and health
of our inhabitants in trustworthy ways using innovative technologies.

This is a letter of intent to show our support for this proposal and intended collaboration.

Sincerely,

Dr. Ir. Wilma Jansen
Senior-researcher

Dr. Jason Pridmore

Date

Thursday, March 10th 2022

Subject

Confirmation to contributing to
SWITCH project

Our reference

Your reference

220310 MV/em

Page

1/1

Appendix

Department

Media & Communication

Visiting address

Dr. Marc Verboord
Burgemeester Oudlaan 50
Van der Goot building
Room M8-05

Postal address

PO Box 1738
3000 DR Rotterdam
The Netherlands

T +31 10 408 2455

E verboord@eshcc.eur.nl

W www.eshcc.eur.nl/English

Dear Jason Pridmore,

I, prof.dr. M.N.M. Verboord, in my capacity of the head of department of Media and Communication at Erasmus University Rotterdam hereby confirm that we will contribute to the Supporting Well-Being through Innovation and Trustworthy Technology for Care and Health (SWITCH) project, as applied for by you, the main applicant, Jason H. Pridmore, Associate Professor at the Department of Media and Communication.

If funded, we commit to allowing you, Jason Pridmore to make use of your already allocated research time as part of our faculty to lead this consortium in connection with colleagues at Erasmus University Rotterdam (ESPhil, ESHPM), Erasmus Medical Centre, and the Technical University Delft. At present, this time amounts to a maximum of 0,4fte already allocated to you for your research work.

I wish you success in your review process.

Sincerely,



Prof.dr M.N.M. Verboord

Head of department of Media and Communication
Erasmus School of History, Culture and Communication
Erasmus University Rotterdam

IV References

- Doekhie, K. D., Strating, M. M. H., Buljac-Samardzic, M., & Paauwe, J. (2019). Trust in older persons: A quantitative analysis of alignment in triads of older persons, informal carers and home care nurses. *Health & Social Care in the Community*, 27(6), 1490–1506.
<https://doi.org/10.1111/hsc.12820>
- Doekhie, K. D., Strating, M. M. H., Buljac-Samardzic, M., van de Bovenkamp, H. M., & Paauwe, J. (2018). The different perspectives of patients, informal caregivers and professionals on patient involvement in primary care teams. A qualitative study. *Health Expectations*, 21(6), 1171–1182.
<https://doi.org/10.1111/hex.12824>
- Institute of Medicine (U.S.), O'Connell, M. E., Boat, T. F., Warner, K. E., & National Research Council (U.S.) (Red.). (2009). *Preventing mental, emotional, and behavioral disorders among young people: Progress and possibilities*. National Academies Press.
- Jaiswal, J., & Halkitis, P. N. (2019). Towards a more inclusive and dynamic understanding of medical mistrust informed by science. *Behavioral Medicine*, 45(2), 79–85.
<https://doi.org/10.1080/08964289.2019.1619511>
- Kramer, R. M., & Tyler, T. (1995). Trust in organizations: Frontiers of theory and research. SAGE Publications. <http://ebookcentral.proquest.com/lib/eur/detail.action?docID=997037>
- LoCurto, J., & Berg, G. M. (2016). Trust in healthcare settings: Scale development, methods, and preliminary determinants. *SAGE Open Medicine*, 4, 205031211666422.
<https://doi.org/10.1177/2050312116664224>
- Nissenbaum, H. (2004). Privacy as Contextual Integrity. *Washington Law Review*, 79(1), 119–158.
- Sauer, M. A., Truelove, S., Gerste, A. K., & Limaye, R. J. (2021). A failure to communicate? How public messaging has strained the COVID-19 response in the United States. *Health Security*, 19(1), 65–74. <https://doi.org/10.1089/hs.2020.0190>
- Xu, J., Le, K., Deitermann, A., & Montague, E. (2014). How different types of users develop trust in technology: A qualitative analysis of the antecedents of active and passive user trust in a shared technology. *Applied Ergonomics*, 45(6), 1495–1503.
<https://doi.org/10.1016/j.apergo.2014.04.012>